



Reach and Capacity of Black Protestant Health Ministries as Sites of Community-Wide Health Promotion: A Qualitative Social Ecological Model Examination

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Abstract

Black communities in the Southeast United States experience a disproportionate burden of illness and disease. To address this inequity, public health practitioners are partnering with Black Protestant churches to deliver health promotion interventions. Yet, the reach of these programs beyond the organizational level of the Social Ecological Model (SEM) is not well defined. Thus, the aim of this study is to understand Black Protestant church leaders' and members' perceptions about the capacity of their ministries to reach into their communities, beyond their congregations, as providers or hosts of health education or promotion interventions. From 20 Black Protestant churches in Atlanta, GA, 92 church leaders and members participated in semi-structured interviews. Grounded theory guided data analysis and a diverse team coded the interviews. Most participating churches had health ministries. Participants saw the boundaries between their churches at the organizational level of the SEM and the broader Black community to be porous. Those who described their "community" as being broader than their congregation also tended to describe community-wide health promotion their church engaged in. They described church-based health fairs as a strategy to promote engagement in their communities. Some participants, particularly those in a health-related profession, discussed visions of how to utilize their church as a site for community-wide health promotion. We suggest these participants may be boundary leaders who can build relationships between public health professionals, pastors, and congregants. Based on the findings, we suggest that church-based health fairs may be effective sites of community-wide health promotion.

Keywords African American · Black church · Social Ecological Model · Boundary leader · Health fairs · Faith-placed

Introduction

Black communities in the United States (US) disproportionately experience illness and disease, necessitating continued effective health education and promotion efforts [1–4]. From 2015 to 2018, 38.7% of Black men and 55.9%

of Black women were obese [5]. The prevalence of Black Americans living with type 2 diabetes is 1.7 times that of white Americans [6]. Additionally about 57% of both Black men and women are living with hypertension [5]. In 2021, Black Americans accounted for 40% of new HIV diagnoses when they accounted for 13.4% of the overall US population [7, 8]. Health disparities have been further exacerbated by the COVID-19 pandemic, where Black Americans have been disproportionately affected by COVID-19 morbidity and mortality [9–15].

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In response to social, economic, and health disparities, Black Protestant churches¹ have a long history of providing individual, organizational, communal, and social level services to Black Americans [19–24], so much so that sociologist E. Franklin Frazier asserted that these churches were the center of Black life in America [25]. The majority of Black Protestant churches in the US have some form of community or social programming [26–29]. Examples include food pantries, cash assistance, health care and screening, voter registration, and social/political advocacy [18, 26, 30, 31]. Sociologists of religion have examined Black Protestant churches through the lens of the emic priest and prophet categories [18, 26, 32, 33]. Priestly functions relate to spirituality and prophetic functions relate to social engagement. Barnes reports that quantitative “models do not support the prevailing sentiment that a spiritual/religious emphasis depresses political or social activism” [26]. It is not easy to categorize churches as solely priestly or prophetic as they can, and do, engage in mixtures of both, as Lincoln and Mamiya noted when they suggested a dialectical approach to examining the orientations of Black Protestant churches [18]. Additionally, while both white and Black Protestant churches in the US engage in community-wide focused activities, Black Protestants engage in more communal and social level activities oriented towards civil rights, policy reform, and the social determinants of health [33, 34].

Addressing the social determinants of health, scholars have utilized church-based health fairs to support communities in addressing non-communicable diseases (NCDs) and HIV [23, 35–37]. Health fairs provide an opportunity to screen for NCDs and sexually transmitted infections, like HIV, as well as provide health education [37]. The faith-placed² nature of these fairs may guide perceptions of community/medical partners as trusted sources of information and care [38, 39]. Although, Schneider and Bolger argue that public health scholars need to critically examine when and how pastors lend trust to outside practitioners and researchers, as this is not a one-sided decision that the congregation inherently accepts but is negotiated [32]. While faith-placed

health interventions are generally reported positively in the literature, health fairs are not ubiquitously attended due to need for childcare or transportation, as well as fears of being diagnosed with an infection/disease in front of one’s community [38–40]. Health ministries, which sponsor health fairs, are often run by women, some of whom have a formal health profession background, but who also face barriers such as volunteer burn out and difficulty accessing evidence-based interventions [38].

Black Protestant churches are ready partners for public health practitioners working in Black communities [35, 36, 41–43]. But these interventions are often targeted only at congregation members, placing them at the organization level of the Social Ecological Model (SEM) [35, 44, 45], despite evidence of the historical and contemporary impact of Black Protestant churches on social policy and reports of successful health fairs. However, Hardison-Moody and Yao describe new connections between Black Protestant churches and community organizations [46]. While many churches in their study received resources from community organizations, some churches both received resources and brought resources to serve community-wide focused health promotion. This is novel because they describe using church resources to bring health promotion to a broader community, in addition to the congregation. Thus, the aim of this analysis is to understand Black Protestant church leaders’ and members’ perceptions about the capacity of their ministries to reach into their communities, beyond their congregations, as providers or hosts of health education or promotion interventions.

Additionally, this analysis examines how congregation members with professional healthcare backgrounds may be boundary leaders. “Boundary leadership is the practice of leadership in the boundary zone, the space in between settled zones of authority, where relationships are more fluid, dynamic, and itinerant” [47]. Where religious leaders and public health professionals both hold different types of authority, boundary leaders can move between the healthworlds they occupy, building bridges of communication and understanding [47–49]. “Healthworlds”³ are made up of all the background and taken-for-granted information of an individual or group and are oriented towards a *telos* of comprehensive well-being [50]. When religious leaders and public health professionals both have goals of helping others achieve well-being, boundary leaders are able to see the differences in what it means to be “well” in these different healthworlds, see the forms of authority that each holds and

¹ We refrain from using the terminology “the Black Church” in recognition of debates among historians of American religion who contest the existence of a unified or historically ontological “Black Church” [16, 17]. We choose the term “Black Protestant churches” because of greater specificity and because it still allows us to categorically conceptualize these churches in the lineage of Lincoln and Mamiya, who think about how conversations and trends can be observed across non-monolithic denominations and churches that are predominantly Black and Protestant [18].

² Campbell et al. differentiate between “faith-placed” interventions which are situated within church buildings and draw on church resources and “faith-based” interventions which also incorporate aspects of the religious tradition into the intervention (e.g., incorporating Bible verses into health education lessons) [35].

³ Germond and Cochrane build their construct of the healthworld from Habermas’ construct of the “lifeworld,” such that the healthworld is understood to be a specific region of the lifeworld [50].

attempts to exert, and can translate between them to build partnerships.

Methods

This inductive secondary analysis examines interviews from the Training and Empowering African American Churches on HIV and AIDS (TEACH) study. TEACH was a larger randomized control trial to study the feasibility and impact of providing predominantly Black Protestant churches in Atlanta, GA, with the previously tested *P4 for Women* HIV prevention intervention [37, 51, 52]. Through TEACH, Emory University Rollin's School of Public Health supported participating churches in hosting HIV education.

Positionalities

TEACH was a large project that brought together a local community advisory board, pastors of predominantly Black Protestant churches, women from participating churches who led education sessions, and a racially diverse study staff. Additionally, the team of study staff and graduate research assistants who conducted, transcribed, and coded interviews were diverse in race/ethnicity, age, gender, and area of graduate study (e.g., health behavior, epidemiology, religious studies).

Authors and study staff took steps to engage in reflexive practice throughout the research process. GMW, a Black woman, and RJD, a white man, conceptualized the study and developed the community advisory board. GMW reflected on how her and the study project director's positionalities as Black women may have helped participants feel supported as they implemented the intervention and were invited to participate in interviews. DNL, a white woman, developed the interview guide used in the TEACH qualitative data collection. Additionally, she oversaw codebook development and supervised graduate research assistants who conducted interviews, transcribed, and coded data. She was intentional about building a team of graduate research assistants who were diverse and could bring new perspectives and interpretations of data. TJF, a white man, with graduate degrees in both sociology of religion and public health coded and analyzed data. His training in the sociology and history of religions in America informed his research questions, data analysis, and contextualization of findings.

Sample Selection

Study staff and community advisory board members jointly identified and recruited churches to participate in TEACH from 2014 to 2016. To participate in TEACH, churches needed to (1) be located in one of the five metropolitan

Table 1 Demographic characteristics of church leaders and program facilitators who participated in qualitative interviews ($n = 92$)

Variable	Total <i>M (SD) or n (%)</i>
Age, years	48.0 (13.3)
Gender	
Women	84 (91.3%)
Men	8 (8.7%)
Education level	
Graduated high school/GED	4 (4.3%)
Completed technical school	3 (3.3%)
Completed associate degree	8 (8.7%)
Started college	19 (20.6%)
Graduate from college	18 (19.6%)
Started/completed masters/PhD	40 (43.5%)
Ever worked as a healthcare professional	
Yes	33 (35.9%)
No	59 (64.1%)
Church staff or leadership position	
Yes	58 (63.1%)
No	34 (36.9%)

Atlanta counties with the highest prevalence of HIV; (2) have consent from the pastor to participate; (3) have a predominantly Black/African American Christian congregation—defined as “all or most attendees [are] Black and the senior religious leaders are Black” [53]; (4) commit to 1 year of participation in TEACH; (5) have one church leader (of any gender), not necessarily a pastor, participate in planning intervention session implementation; and (6) have at least two women who were church leaders or members participate in planning and facilitating the intervention sessions, as the education sessions were for women specifically. All church leaders and members participating in TEACH were invited to participate in optional interviews and self-selected to volunteer.

Population

A total of 130 church leaders and members participated in TEACH and 93 (71.5%) volunteered to participate in the interviews analyzed here (Table 1). Every church participating in TEACH had at least one leader or member participate in interviews. One participant was excluded from analysis due to poor audio recording quality. Participants represented both church leaders (63.1%) and members (36.9%) who helped facilitate *P4 for Women*. All participants were Black/African American. Participants' ages ranged from 20 to 83 years with a mean of 48.0 years. Most participants were women (91.3%). Most participants were well educated with over half having graduated college or started or completed

Table 2 Demographic characteristics of Black Protestant congregations who participated in the TEACH program ($n = 19$)¹

Variable	Total <i>M</i> (<i>SD</i>) or <i>n</i> (%)
Denomination	
Methodist	7 (36.8%)
Baptist	6 (31.6%)
Non-denominational	5 (26.3%)
Pentecostal	1 (5.3%)
Active church membership size	4983 (9343.2)
Church age, years	80.3 (58.3)
Tenure of current pastor, years	18.1 (11.4)
Offer transportation	
Yes	14 (73.7%)
No	5 (26.3%)
Have multiple locations	
Yes	7 (36.8%)
No	12 (63.2%)
Have a health ministry	
Yes	16 (84.2%)
No	3 (15.3%)

¹One church did not provide the requested demographic information about their congregation

graduate/professional school. About one-third of participants had worked in a healthcare profession (35.9%). Many participants attended mega-churches (45.7%), about one-third attended large churches (32.6%), and a few attended medium-sized churches (11.9%) [54].

Of the 20 participating churches, 19 provided information about the demographic characteristics of their congregation (Table 2). All participating churches were predominantly Black and Protestant. Denominationally, churches were roughly split between Methodist (primarily including African Methodist Episcopal and Christian Methodist Episcopal), Baptist, and non-denominational churches (these are Christian churches not institutionally tied to a denomination), with one Pentecostal church participating. Many churches were classified as mega-churches, having over 2000 active church members ($M = 4983$, $SD = 9343.2$) [54]. Many churches had pastors with stable tenures ($M = 18.1$ years, $SD = 11.4$ years).

Data Collection

Church leaders and program facilitators participated in one-on-one semi-structured interviews conducted by study staff or graduate research assistants. Interviews occurred March 2015–February 2016, before participants had received training about the *P4 for Women* intervention. Participants chose the location of their interview, with most taking places at the participant's church, a study staff member's office, or a quiet

public location. Interviews lasted between 20 and 120 min, with most taking about 60 min, and were audio-recorded. Interviews included questions related to initial attitudes towards the TEACH study, existing health education offerings within the participant's church, opinions about sexual and reproductive health education in Black churches in general, stigmatized and taboo topics in predominantly Black churches, and personal religious upbringing. All participants provided written informed consent to participate and the Emory University Institutional Review Board approved all study procedures prior to data collection. No compensation was provided for participation in interviews.

Data Analysis

As part of TEACH, 92 interviews were transcribed verbatim by trained graduate research assistants and study staff. Grounded theory guided data analysis and inductively identified themes were used to create a structured codebook with 67 parent codes (Table 3 presents entries for codes utilized in this analysis). Teams of three to four graduate research assistants or study staff thematically coded interviews using NVivo. Cohen's kappa measurements were generated using NVivo to compare inter-coder reliability; codes with Cohen's kappa < 0.8 were coded to agreement. To code to agreement, coders would meet and review all coded segments, collectively agreeing on how to apply codes. In addition to thematic coding, interview participants were categorized by whether they had a health profession background. This was not asked about in initial demographic questions. Thus, a team of two graduate research assistants examined transcripts for participants' self-identification or description as ever having worked in healthcare. They descriptively categorized participants for ever having worked as a healthcare professional and jointly came to agreement on categorization. Examples of working as a healthcare professional included, but were not limited to, working as a nurse, county health department employee, licensed professional counselor, or health educator.

This secondary analysis is derived from observations of participants describing health fairs within discussion of congregation health education and the importance of communities being educated on HIV and AIDS. Codes from the larger codebook utilized in this analysis included Current Health Education Offerings, Importance of Educating Others, and Definition of Community (Table 3). TJF reviewed segments in the Current Health Education Offerings and Importance of Educating Others codes and organized them across the SEM. Because participants used the term "community" in a variety of ways that did not always line up with the definition of the community level of the SEM, TJF first reviewed each transcript segment coded as Definition of Community. If participants described their congregation as

Table 3 Definitions and examples of codes used in analysis

Code name	Code definition	Example
Current Health Education Offerings	What health education is available or is not available in the church and community	“As far as health, you know, we have like the usual health fairs, like as far as cholesterol, high blood pressure, diabetes, prostate checkups, mammograms, breast cancer, just that kind of things. And immunization for the kids, general physical, yearly physicals you know for males and females....” (woman, 44, church administrator)
Definitions of Community	Participant’s own definition of what their community is	“Although it’s information out there everywhere, I still believe that a lot of times in our community – when I say our community, I say the black community – we tend to believe that it’s a certain group or demographic that’s exposed to HIV.” (woman, 43, church member and community HIV educator)
Importance of Educating Others	Discussions about what it means to inform their perceived community and pass down knowledge	“So, if you’re not giving them the education they need, then they’re leaving out and where you had the opportunity. Like if you have the opportunity and the platform then utilize it. For the betterment of the community. Especially when something like [HIV] is really tearing down communities.” (woman, 31, biblical school facilitator)
Healthcare Experience	Previous or current healthcare experience that the participant self-identified	“Well you know I worked in a STD clinic and of course you know I was a public health nurse and we have to educate ourselves on these things.” (woman, 60, director of education and former public health nurse)

their community, then discussions of engaging community members in health education were placed at the organization level of the SEM because the participant was using the term to describe their congregation, which fits at the organizational level of the SEM. For the purpose of this analysis, the authors define community as a population of people sharing a racial/ethnic identity living within a given geographic area. In practice, this refers to individuals who identify as Black/African American and live within the Atlanta metropolitan area. Thematically coded segments were stratified by the descriptive healthcare professional classification and definitions of “community” for analysis. Comparisons were made between participants with healthcare backgrounds and those without healthcare backgrounds as well as those who inductively defined “community” as being their congregation, their racial/ethnic group, or their geographic location. The unit of analysis was all coded segments per participant.

Results

Across the 20 participating churches, 80% had a ministry that offered health-related education or services. These ministries varied in their form, function, and staffing, although most were implemented by women who volunteered. Participants who had previous experience working as healthcare

professionals appeared particularly confident in the church’s ability to engage in community-wide health promotion. Two types of boundaries are examined in these results: (1) the boundaries between the organizational and community levels of the SEM, and (2) the boundaries between religious and public health healthworlds. Table 4 shows the impacts participants described Black Protestant health ministries having at the individual, organizational, community, and society levels of the SEM.

Porous Boundary Between Organizational and Community Levels

Within the Definitions of Community code, participants utilized several recurring themes to identify their community, including race (e.g., Black/African American community), geographic location (e.g., neighborhood, Atlanta), and church membership. Thinking beyond the church, one participant articulated the difference, asking,

In my community? I guess it’s kind of a broad question. Cause, it depends on what defines my community in terms of where I live? Or in terms of my heritage and my background? So those are two different things.
(woman, 32, church member and public health professional)

Table 4 Black Protestant health ministries (BPHMs) can reach across the SEM and impact health behaviors and promotion of individuals, congregations, wider communities, and society

SEM level	Impacts	Example
Individual-level	• BPHMs can give health education materials to individuals in need. • BPHMs can connect individuals with resources in the community (e.g., services for individuals experiencing homelessness).	• [The mission of the health ministry is] “to provide education and referrals, and help people with any health issue that they may have.” (woman, 63, church secretary and licensed clinical counselor)
Organizational-level (congregation)	• BPHMs can hold health education classes and health promotion programs for their congregation.	• “I helped facilitate a group called the Biggest Loser Challenge. We were helping individuals lose weight as a part of the church they joined and we would work out with them every Tuesday and do things that nature.” (woman, 23, health ministry representative)
Community-level	• BPHMs can host health fairs that are open to the community. • BPHMs can bring additional health organizations to the community through health fairs. • BPHMs can also reinforce stigma about sexual health, including HIV testing.	• “We do a health fair every year, so we want to be able to provide them all types of information on any particular health issue. So I think it would only enhance what we’re already doing and put us more visible in the community.” (woman, 51, church member and substance abuse counselor) • “A lot of churches that I’ve been to, large or small, they’re doing a health fair in this room, all the HIV testing is down the hall, ‘go down the stairs, around the building, knock on the door three times, give the password to do HIV testing’ which again, that stigmatizes the HIV testing.” (man, 48, associate minister of HIV/AIDS)
Social-level	• BPHMs can be places where individuals gather to learn and put that learning into action, including action at political and policy levels.	• “So, I think being knowledgeable, when we know better we usually do the better things. And that’s from being in church and then all the way up to political...like policy... all kind of... any- every area- any area that we’re involved in that we can basically impact, it’s beneficial.” (woman, 39, associate minister)

These understandings of community informed how the authors conceptualized “community.”

Among those who named their community as being broader than their congregation, many perceived churches to be important organizations. They also saw the boundary between the organizational level congregation and the community level to be porous. One participant articulated a common perception, stating, “I thought that it was a good process of reaching the community only because the church is like, has always been in the south, the standpoint of the community” (woman, 51, church member and HIV specialist). These participants described how community members could attend various churches, although many had a primary church. They described predominantly Black Protestant churches as important community spaces because they hold significance for many community members, even if they do not attend that specific church.

In addition to reaching out from the congregation to the community, some participants also saw churches as a place where community members could join to discuss important topics. This participant stressed how church members are part of broader communities, making churches community spaces.

Like I said these days that’s a big issue in the community so I think that it should at least brought up and discussed in the church setting. Because as church members we are part of the community as well so it only makes sense. (woman, 26, church member and pharmacist)

She emphasized the importance of talking about important community issues in the church. Because of the perceived importance of the church within communities, participants described how the church needs to be a place where health can be talked about and information can be received by community members whether they are part of the congregation or not.

Seeing “Community” as Racial/Geographic and Seeing Community-Wide Health

Within the Current Health Education Offerings code, many health ministries primarily served their immediate congregations. When identified, community members were often listed as secondary recipients of health ministry services. A Health Ministry Co-Director shared,

“That’s all congregants. It doesn’t take into consideration anything other than they’re a member of the church... or even nonmembers of the church. We give information to everybody” (woman, 54). Individuals who were not members of the church would be welcomed and could draw on religious social capital in the form of information and services. Yet, they were not described as being a population that health ministry leaders had in mind when planning or executing health promotion interventions. Overall, most ministry activities described were for the benefit of congregation members.

Yet, participants who described their “community” in terms of race/ethnicity and/or geography within the Definitions of Community code also tended to describe community-level health education in the Current Health Education Offerings or Importance of Educating Others codes, whereas those who described their “community” in terms of their congregation tended to describe organizational-level health education. Below are examples of definitions of community and SEM level of health education they describe.

First, a woman (72) who was a leader in her church.
Definition of Community - Congregation

Interviewer (I): Okay. What about in your community, do you feel that there is a need for more sexual and reproductive health programs in your community in general?

Participant (P): In my church community, yes. Yeah, we need to be educated more.

SEM Level of Health Education – Organizational Level

I: There’s something else that you just mentioned... oh you said sex education classes? Do you’ll have classes here or?

P: In our youth department.

I: Okay.

P: They’re taught about sex education, it’s a whole, it’s an engagement class.

She was passionate about educating youth on sexual and reproductive health because several youth in her congregation were living with HIV and because she had been married young, at 17 years old. These experiences shaped her focus when she was asked about sexual health education.

Second, a woman (53) who was the Vice-President of the Lay at her church and former healthcare professional.

Definition of Community – Race/Ethnicity

I: Did you know that HIV was an issue in your community?

P: Did I know that it was? For African Americans? Yes.

SEM Level of Health Education – Community Level

The health fair that we have every year, everybody approves of that because we do offer HIV screenings, diabetes- we offer various screenings for people in the community as well as in the church.

This participant saw her community as the broader Black/African American community, and in a later part of the interview also discussed her “community” in geographic terms. She also made a distinction between the people in the “community” and the people in the church as being welcome at the church health fair.

There was not a discernable difference between how those with or without a healthcare professional background identified their “community” nor a difference between if they described only organizational or organizational and community-level health education efforts.

Health Fairs Reach Across the Boundary to the Wider Community

Health fairs often appeared at the intersection of the Current Health Education Offerings and Importance of Educating Others codes. Health fairs were a way for church health ministries to reach beyond their immediate congregation, across the SEM boundary to their broader community. Most commonly, they were an annual way to engage congregation and community members in a variety of health topics that were relevant to them. A First Lady (the head pastor’s wife) shared,

Usually most of the [ways people were drawn in to talk about health] would be connected with our health fair, with the church, and for the community. Like back in early February, we had a women empowerment fair. We had information on health and wellness, your blood pressure, glucose, the routine health fair. (woman, 55)

Health fairs were versatile because they hosted a variety of informational booths and services. Often, the churches relied on outside support in putting on these fairs. The Director of Activities Department at his church, and a former mental health social worker shared, “The Alpha Kappa Alpha’s... provide all the vendors [for the annual health fair]. And... They reserved the mammogram mobile last year after the first one, so they’re on schedule to be there again this year” (man, 73). Participants described health fairs as events that were hosted by churches but are open to the community as a whole. A former public health nurse and Director of Education at her church described this saying,

You would have this big health fair in the community and you know the homeless would come and get clothes, and get shade, get a bath, we'd check their blood pressures, and all of this and they'll get food, and ladies get their hair done. (woman, 60)

She also highlights how her church uses health fairs as an opportunity for community members, including the most vulnerable, to receive a variety of health services in one location and at one time.

Occasionally, participants described health work in the community and with marginalized groups (e.g., those who are homeless) as part of evangelism ministries. For some participants, their faith encouraged them to engage in evangelism, which they combined with community health outreach efforts, seeing these efforts as an opportunity to practice tenets of their faith related to evangelism and the provision of social services.

My pastor, he has wanted to do something like this [HIV education and prevention program] for the community. We're always giving to the shelters around here – evangelizing, you know, trying to figure out what's going on in the neighborhood. And that's one of their concerns is, help for HIV patients ... So they're wantin' the church, you know, to inquire about resources for those type of things, so that's something that's on his agenda. (woman, 44, church administrator)

Evangelism provided both motivation and a place within the church ministry structure to plan community health programing.

Healthcare Professionals as Boundary Leaders

While participants who did not have a health profession background sometimes described activities that their church was putting on, participants with a health profession background were often making connections and had a vision for the future. Healthcare professional experience was described as a strength for those working in church health ministries because it allowed participants to engage in work and build partnerships.

And so it's very helpful that we have people who are professionals. I mean, these people who actually are doing the work, have their doctorates in, you know, from the medical university... Or, you know, we have people who can actually do the work. So although we are small, we have high powered thinkers who actually have the impetus and the desire to give people the information that they need. (man, 48, minister and HIV prevention and counseling experience)

Within the Current Health Education Offerings and Importance of Educating Others codes, these participants expressed not only a desire to reach across the boundary between the organizational and community levels, beyond their congregation to their broader communities and community partners, but also a vision of what was needed in order to do this successfully.

Some participants discussed making the wider community aware of the health fair through advertising. A participant shared how her congregation sought to raise awareness via advertising, saying,

Well we have a health fair, and we invite the community out. So we normally get a decent turnout if we can get enough radio time to let them know that we having a health fair and then they will come out. (woman, 47, church member and nurses aid)

Lack of advertising to the wider community was a barrier to wider community engagement with health fairs and other health events open to the public, such as seminars on health-related topics.

Most participants expressed the need to collaborate with experienced health educators, who can bring expertise, skills, and resources to health ministry events and programs. Participants without a health profession background saw outside organizations providing them with resources, commonly as teachers to come and assist their churches. A church administrator implored, "You have to show us, show us the way" (woman, 44). Conversely, participants with health profession backgrounds saw outside organizations as partners in their work within both the church and the wider community. They were able to connect their congregation with outside public health resources for the benefit of their wider community. A Health Ministry Co-Director and Public Health Nurse shared,

We used to do diabetic management. We worked with [the] Cancer Society. We worked with the youth in doing health fairs. We have worked with AID Atlanta doing screenings in schools right around. We do third Sunday ministry for our homeless population, blood pressure and diabetic screening. And we do the same thing for the regular congregation, we have on the third Sunday we have... We do a speaker come talk about some reproductive health as far as education is concerned. (woman, 61)

Some partnerships benefited from participants who were able to think both theologically/spiritually and physically about health. A nurse and health ministry co-director shared the following when asked if she would be comfortable leading *P4 for Women* in her church.

Sure. My eldest son is openly [HIV] positive... He taught me and I taught others. And when I worked with the [County Health Department], I took a course there for teaching and then we took a course with a seminary. So I'm comfortable... But as far as personal feelings for me it's just like cancer, or diabetes, or hypertension. It isn't a choice. And you can't say "That I did this" or "you got this." No... it happened. (woman, 61)

This woman was able to remove stigma from HIV and think about it like other infections and NCDs. Doing so, she bridged the boundary between the healthworlds of congregations and healthcare professionals.

Discussion

While churches are often placed at the organizational level of the SEM [35], our research suggests that some Black Protestant health ministries are interested in being sites of community-level health promotion. Hardison-Moody and Yao have described linking North Carolina churches to community-wide health promotion programs where many churches drew on the resources of community partners, but some also joined public health practitioners seeking to improve community-wide health [46]. Seeing churches as capable of engaging in community-wide health education and promotion opens up a new landscape of possibilities for community-engaged research and practice. Our work corroborates this description in Georgia, showing church leaders' and members' interest in engaging in community-wide health promotion, describing their needs for this work, and identifying church-based health fairs as a possible mode of community health promotion. Our study also expands on previous work by examining (1) how understandings of "community" encompassing the church community or racial/geographic community may link to interest in community-wide health promotion and (2) how those with current or previous work in a healthcare profession understand the role of Black Protestant churches in community health efforts.

This study indicates that some Black Protestant church leaders and members see their health ministries as being capable of reaching into Black communities to provide health services. Murphy et al. note that Black Protestant churches have long been a "key area" to host health promotion in Black communities, but question whether it is still relevant, due to changes in church attendance demographics [41]. Yet, in this study, Black Protestant church members perceive the boundaries between the congregation and community to be porous and still want to engage community members who are not in the pews. Similar to participants here, Warren and Charles also describe Black Protestant

churches as spaces that welcome community members to join them, even if they do not adhere to that church's theology [36]. These perspectives may be informed by Black Protestant churches' historical role in shaping Black community spaces, the provision of social services, and the experience of Black culture in the US [18, 25]. Participants perceived Black Protestant health ministries as having the socio-cultural significance and religious social capital needed to be sites of community-wide health promotion.

Participants who saw their "community" as larger than their congregation, often in racial or geographic terms, also saw the ways that their congregation invited community members to health education and promotion events. Yet, this did not necessarily make these participants boundary leaders. Church members with previous health experience had the desire and vision to engage in community-wide health education and promotion by drawing on the healthcare experience within their congregation and forming partnerships with outside organization. Doing so, they acted as boundary leaders, bridging "settled zones of authority" [47] between church leaders and public health professionals. Religious and public health healthworlds often understand "health" in different ways. Some participants in this study articulated how "health" included the spiritual, which has also been previously documented [18, 22, 35, 36, 55]. So, having a congregation member who can navigate those two healthworlds—seeing "health" as physical, mental, and social well-being or seeing "health" as physical mental, social, and spiritual well-being⁴—and translate across them is an important health asset⁵. We suggest that these boundary leaders can help public health practitioners initiate and engage in conversations about the need for community-level health promotion, and the ways that the Black Protestant churches can be a part of these interventions. We also suggest that as members of the congregation, healthcare professionals, and the wider community, boundary leaders may be instrumental in building the trust and facilitating negotiations over trust between public health professionals, pastors, and congregants that Schneider and Bolger describe as essential to health education and promotion efforts [32]. Yet, boundary leaders often need a catalyst [47–49]. Thus, to tap into the vision and power of boundary leadership, these individuals need to be brought to the table and given resources to build relationships. We also suggest that encouraging them to consciously thinking about their congregation as a part of a larger geographical/racial community may be a formative step that helps them thinking about their congregation's

⁴ Here, we align the public health healthworld with the World Health Organization definition of "health" [56].

⁵ We draw on Gunderson and Cochrane's work on "Religious health assets" here [47].

ability to engage in community-wide health education and promotion.

Limitations and Areas for Future Research

The inductive qualitative nature of this secondary analysis is a limitation because we did not systematically ask participants questions related to these research questions, nor did interviewers systematically probe for more detail when the themes explored here were discussed [57]. Thus, we are limited in the theoretical interventions we can make. Yet, the findings of this formative study pose questions for future research, which can elucidate these themes in greater detail, explore relevant themes that did not inductively emerge in our research, and make deeper theoretical interventions. Future research can examine the capacity and needed resources of Black Protestant churches to host health fairs, as well as the role of boundary leaders in facilitating the partnerships necessary to host health fairs. Qualitative evaluations of boundary leaders' narratives of being catalyzed towards action in their congregation health ministries and community may be instructive. The question of how these health fairs and health ministries reach out into wider communities also needs scholarly attention. Additional research on the perspectives of Black Americans who are not members of congregations can also provide data that will help examine these questions from multiple points of view. Additionally, evaluation of health fairs will continue to be a necessary part of understanding church-based health interventions targeted towards a wider community. Campbell et al. suggest that this is an important step, as many church-based health promotion programs are not fully evaluated, leaving questions about the measurable impacts of these programs at congregation and community levels [35].

As is relevant to all qualitative research, the themes here cannot be generalized to all communities [57], including all Black communities in the Southeast US. Several important sub-populations of Black communities were absent, under-represented, or unidentified in these interviews including Black Americans who are experiencing low socio-economic status or who are living with chronic diseases. Because classification as a healthcare professional relied on unprompted self-identification during the interview, there may have been participants with relevant experience who were not classified as healthcare professionals and were misplaced in stratification. We also had low participation from pastors at some participating churches, as most participants were ministry leaders or church members; thus, pastors' perspectives may be absent from this work. In a similar vein, the majority of interview participants were women; thus, it may be useful to further explore the perspectives of Black men. Our sample was also populated by churches from the Atlanta metropolitan area; thus, the generalizability of the results and their

implications to rural areas will need to be further explored and articulated. Finally, we did not have data from Black community members who were not part of a congregation. Their perceptions will be needed to fully understand this issue.

Conclusion

We assert that Black Protestant churches and public health professionals can reciprocally benefit from partnerships that draw on churches' religious social capital and public health professionals' access to evidence-based community health interventions when seeking to engage in community-wide health promotion. Our participants demonstrated interest in this type of partnership when they framed "community" in geographic/racial terms, instead of congregational terms. Additionally, participants with experience working in a health-related field also demonstrated having a vision of how to engage in productive partnerships. We suggest that health fairs may be a good starting point for partnerships because our participants described them as a mode of moving through the porous boundaries that they perceive between congregations at the organization level and the larger Black community.

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Data Availability Data are available upon reasonable request.

Code Availability Codes are available upon reasonable request.

Declarations

Ethics Approval Emory University's Institutional Review Board approved all study procedures prior to data collection.

Consent to Participate Informed consent was obtained from all individual participants included in the study.

Consent for Publication Informed consent was obtained from all individual participants included in the study.

Competing Interests The authors declare no competing interests.

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